[redacted] False, indisputable

[redacted] False, because I/Gie City lied/withheld information

[redacted] False/inaccurate, disputable

[redacted] Subjective - projecting their values onto my/Gie City's life

Government of South Australia
SA Health, Operations
Adelaide Metro Mental Health Directorate

27th June 2013

Eastern Community Mental Health

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Eastern Community Mental Health Centre
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Dr Wong
Tusmore Surgery
103 Tusmore Avenue
Tusmore SA 5065

Dear Dr Wong

Re: Giles **CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Olanzapine 15mg nocte
Citalopram 10mg mane

Giles Cullity was reviewed on the 25th June 2013 at Eastern Community Mental Health Service at Tranmere with his mother Adrienne and Inge.

Personal Profile
Giles is a 19 year old single man who lives at Leabrook with his parents [redacted] and his three brothers, [redacted], [redacted] and [redacted].

Giles is currently under home detention for an assault committed earlier in the year when he was living on the streets.

He is on no Centrelink Payments and receives no income.

He had started a bridging course to assist in gaining entrance to an engineering course at university but dropped out at the end of last year. He is not currently studying and he is not doing any work.

History of presenting complaint
Giles had had a recent admission to C3 where he was given a diagnosis of Bipolar Disorder/Schizoaffective Disorder. He had been seen at home by rapid response team with police and mental health team after expressing suicidal ideation and intent. He was admitted to Margaret Tobin Centre in a closed ward and then transferred to C3 at the RAH. He has been out of hospital for three weeks.

He states that the suicidal ideation he had prior to hospitalisation was the consequence of trauma induced by a video he had watched. He did not want to discuss this further.

He said that although he still had intermittent suicidal he no longer saw any reason to act on these ideas and he said there was no chance of him acting on these thoughts. He is stopped by thoughts of the effect this would have on his family. He does however continue to question the meaning of life.

At present he said he was feeling "alright" or "average". He denied any need for psychiatric intervention. He said his mood is normal and he is not depressed or overly happy. He is not worried about anything. He was preoccupied with his vision for the future and his role in sorting out some issues for the world. His concentration is good. He is able to maintain interest and gain enjoyment. His energy levels are fair. He does report a reduced speed of thought from his medication. His motivation is variable. He does not feel hopeless, worthless or guilty.

In contrast to this however there are reports that his self-care has been diminished at home and he had been keeping odd hours going to sleep between 0500 and 1500.

He is keen to achieve some progress for the world in several areas. This includes a new web browser and organisation of information on the computer which will be beneficial for everybody. He does not have any IT programming skills but has concepts. He also has other plans but did not discuss these further. He said his family was not supportive of his plan and want him to go to Uni and progress with his education. His has had the idea of the organisational system came into his head last year. He needs to rely on someone else to put his ideas into practice.

He denied any other psychotic symptoms. He denies hallucinations. He denied thought insertion, thought withdrawal, thought broadcast or thought echo. He denied delusions of reference from TV or radio and he denied any persecutory beliefs or religious beliefs.

His sleep was probably excessive but he said it was partially refreshing. He felt a bit hung-over at the end of his sleep and he attributed this to medication. He has been sleeping between 0500-1500 and he did not want to change this. His appetite was normal. His weight is 82kg and with a height of 182cm his BMI is 25. His bowels are regular. His libido is normal.

He denied any anxiety. He denied features of post-traumatic stress disorder other than being traumatised by the video that led to him feeling suicidal. He said he had not been a victim of any assaults prior to the events of a few months ago which hd led to him being charged.

He has issues with body image with concerns about his weight and muscle bulk. He said at one stage he was exercising excessively and this resulted in a problem with is muscles becoming weak and he now had to re-train his muscles.

He denied any gambling problems.

He denied any features of obsessive compulsive disorder. He did admit obsessional thought processes. For example his preoccupation with his plans for the computer programme and his preoccupation with exercise when he was spending large periods of time each day preoccupied with his fitness.

He said he had had three very brief episodes of feeling very positive. One episode had been after performing in a concert and then when thinking about his revelations for computer ideas. These had occurred without medication or drug ingestion.

He said he has been taking 15mg of Olanzapine a day Escitalopram 10mg a day which made him feel a bit tired. He did not see them to be of any great benefit and had missed a number of doses in previous weeks. He said he was now compliant.

He does not smoke cigarettes. He has only ever used alcohol once to the point of drunkenness and he does not use illicit substances.

Other History
He denied any medical history of note. He has had contact with adolescent psychiatric services at CAMHS between the ages of 16 and 18. He had seen a number of psychiatrists including Simon Hein, Sally Tregenza and Michael Schirripa in addition to a psychologist for depression and suicidal ideation. It had been of some benefit. He had also then had this recent admission to Margaret Tobin Centre at FMC and C3 at the RAH.

He has had several forensic issues. He had an "assault police" charge following an incident on "school muck up day". There was an assault causing grievous bodily harm recently for which he was due to face court soon for which he has been on home detention.

There is a history of psychiatric illness in both sides of the family with Bipolar Disorder, Anxiety and Depression. There had been one completed suicide in a second cousin.

There is a family medical history of diabetes and heart problems.

He described his parents in positive terms. He found it difficult to accurately describe their personalities. His father is a professor of philosophy and hi mother is a music teacher. He said he had a good relationship with his siblings and said there was no particular conflict.

He was born in Scotland and came to Australia when he was 8 years old. He said he had no problems with the relocation. He found school tolerable. He ended up at Marryatville High school and had some friendships. He was no longer in contact with any friends from school. He has had no girlfriends and did not want one at present. He denied any learning difficulties. He had apparently done reasonably well until about year 12 when he became preoccupied with weight issues and this lead to him dropping studies and not attempting to matriculate. He has subsequently tried a bridging course into university. He had plans to be an engineer. Two of his siblings are studying engineering.

Mental State Examination
He was a tall man with short hair dressed in a tracksuit. He has slowed movements but there was no evidence of agitation or tremor. His conversation was normal in rate but he found it difficult to elaborate detail in a number of issues eg his computer plans. He was also very guarded about some of his beliefs. I was unable to fully explore his body concerns and the distressing video which had made him feel suicidal and continued to distress him. His affect was somewhat restricted but reactive and appropriate at times. He denied any perceptual abnormalities. His cognition was grossly intact. His insight was limited and his judgement was impaired. Rapport was superficial.

Assessment
Giles is 19 year old man with a history of mood disturbances and decline in function over several years. He has had prominent depression and suicidal ideation at times. He has odd ideas about his body and is focussed on computer issues. His beliefs could not be fully explored but appear to be overvalued and probably psychotic. The diagnosis is not entirely certain but has been conceptualised as schizoaffective disorder. He requires further treatment.

Management Plan

1. I have suggested continuing on the current treatment but suggest he take all his Olanzapine prior to bed
2. I will review in 4 weeks after my leave
3. Continue to have mental health team support
4. I have suggested he attempt to change his sleep pattern to a more normal pattern

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely



Dr Phillip Kneebone
Consultant Psychiatrist

Government of South Australia
SA Health, Operations
Adelaide Metro Mental Health Directorate

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23 July 2013

Dr Wong
Tusmore Surgery
103 Tusmore Avenue
Tusmore SA 5065

Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Olanzapine 15mg nocte
Citalopram 10mg mane

Giles Cullity was reviewed on the 23 July 2013 at Eastern Community Mental Health Service.

At Present
Giles is currently under home detention for an assault committed earlier in the year when he was living on the streets. He is spending time at home working on his computer project. He says his court case could take until the end of the year. He will therefore be at home for the rest of the year. His mother reports he still keeps to himself.

At present he said his mood is "average". He is not depressed. He is not worried about anything. His concentration is good. He is able to maintain interest and gain enjoyment. His energy levels are fair. His motivation is reasonable. He does not feel hopeless, worthless or guilty.

He denies suicidal ideation or intent. He questions the meaning of life. He is keen to make a contribution to the world.

He denied any other psychotic symptoms. He denies hallucinations. He denied thought insertion, thought withdrawal, thought broadcast or thought echo. He denied delusions of reference from TV or radio and he denied any persecutory beliefs or religious beliefs.

His sleep has normalised a little. He now sleeps from 0100 to 1200. He is no longer hung-over at the end of his sleep. His appetite was normal. His weight is stable at 82kg. Height 182cm. BMI is 25. His bowels are regular. His libido is normal.

He denied any anxiety. He denied features of post-traumatic stress disorder. He denies any abnormal preoccupation with his body or muscle mass.

He said he has been better taking all the Olanzapine at night.

He does not take any licit or illicit substances.

Mental State Examination
He was a tall man with short hair dressed in a tracksuit. He has slowed movements but there was no evidence of agitation or tremor. His conversation was normal in rate. His affect was somewhat restricted but reactive and appropriate at times. He denied any perceptual abnormalities. His cognition was grossly intact. His insight was limited and his judgement was impaired. Rapport was superficial.

Assessment
Giles seems relatively stable at present with his mood and thought processes.

Management Plan
1. Continue current treatment
2. Review in 4 weeks
3. Care coordinator to look at activity options

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely



Dr Philip Kneebone
Consultant Psychiatrist

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13 August 2013

Dr Wong
Tusmore Surgery
103 Tusmore Avenue
Tusmore SA 5065

Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Olanzapine 10mg nocte
Citalopram 10mg mane

Giles Cullity was reviewed on the 17 September 2013 at Eastern Community Mental Health Service.

At Present
Giles is still under home detention and has a court case possibly in October. He did not want to give us a copy of the forensic psychiatrists report. He is unsure about the future as the court case is still pending and his lawyer has not been very willing to give a prediction of outcome. He does not like home detention.

He is still spending all his time working on his computer project and other interests. He is happy with his progress. He does not have any visitors. He has limited interactions with his family. He has occasional meals only with his family. He says he has nothing much to say to them.

At present he said his mood is "fine". He is not depressed. He is not worried about anything. His concentration is good. He is able to maintain interest and gain enjoyment. His energy levels are fair. His motivation is reasonable. He does not feel hopeless, worthless or guilty. He denies suicidal ideation or intent. He denies any intent to harm others.

He denied any other psychotic symptoms. He denies hallucinations. He denied thought insertion, thought withdrawal, thought broadcast or thought echo. He denied delusions of reference from TV or radio. He denied any persecutory beliefs or religious beliefs.

He continues to have about 9 hours sleep but in no particular pattern. His appetite was normal. His weight has reduced to 79kg. Height 182cm. BMI is 24. His bowels are regular. His libido is normal.

He denied any anxiety. He wants to see a physio about his "inhibited" muscles. He feels his posture is not ideal but he has not done any exercise. .

He does not take any licit or illicit substances.

Mental State Examination
He was a tall man with short hair dressed casual clothes. He has slowed movements but there was no evidence of agitation or tremor. His conversation was normal in rate but he was not inclined to provide much detail about his life or thoughts. His affect was somewhat restricted but reactive and appropriate at times. He denied any perceptual abnormalities. His cognition was grossly intact. His insight was limited and his judgement was impaired. Rapport was superficial.

Assessment
Giles seems relatively stable at present with his mood and thought processes. He has noticed a small change with the reduced olanzapine.

Management Plan
1. Continue olanzapine 10mg nocte
2. Review in 6 weeks after my leave

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely



Dr Phillp Kneebone
Consultant Psychiatrist

Government of South Australia
SA Health, Operations
Adelaide Metro Mental Health Directorate

28 October 2013

Eastern Community Mental Health

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Dr Wong
Tusmore Surgery
103 Tusmore Avenue
Tusmore SA 5065

Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Olanzapine 10mg nocte
Citalopram 10mg mane

Giles Cullity was reviewed on the 28 October 2013 at Eastern Community Mental Health Service. He was seen for part of interview with his father Garrett. His behaviour has been largely controlled but he expresses his frustration at times. His medication is monitored loosely but is suggestive of compliance.

At Present
Giles is still under home detention. His lawyer has said he is not fit to instruct him and this has been supported now by Dr Raeside. He is now seeing another psychiatrist in January. He still does not want to give us a copy of the forensic report. He has a directions hearing in late January.

He is still spending all his time working on his computer project and says he is making slow progress. He believes other people could design something similar and he could be leapfrogged. He is trying to be more productive. He would like to sort out his fitness regime and his body building plans. He is still worried about muscle activation but has not had any assessment eg EMG. .

He does not have any visitors and does not encourage contact as he does not like the distractions. He still has limited interactions with his family. He has occasional meals only with his family. He says the family are not upset with him.

At present he said his mood is normal. He is not depressed. He is not worried about anything. His concentration is good. He is able to maintain interest and gain enjoyment. His energy levels are fair. His motivation is reasonable. He does not feel hopeless, worthless or guilty. He denies suicidal ideation or intent. He denies any intent to harm others.

He denied any other psychotic symptoms. He denies hallucinations. He denied thought insertion, thought withdrawal, thought broadcast or thought echo. He denied delusions of reference from TV or radio. He denied any persecutory beliefs or religious beliefs.

He continues to have about 9 hours sleep but in no particular pattern. His appetite was normal. His weight is stable at 79kg. Height 182cm. BMI is 24. His bowels are regular. His libido is normal.

He does not take any licit or illicit substances.

Mental State Examination
He was a tall man with short hair dressed casual clothes. He has slowed movements but there was no evidence of agitation or tremor. His conversation was normal in rate but he was not

Inclined to provide much detail about his life or thoughts. His affect was somewhat restricted but reactive and appropriate at times. He denied any perceptual abnormalities. His cognition was grossly intact. His insight was limited and his judgement was impaired. Rapport was superficial.

Assessment
Giles seems relatively stable at present with his mood and thought processes.

Management Plan
1. He will continue his current treatment
2. He will be reviewed in 6 weeks

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Philip Kneebone
Consultant Psychiatrist

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11 December 2013

Dr Wong
Tusmore Surgery
103 Tusmore Avenue
Tusmore SA 5065

Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Olanzapine 10mg nocte
Citalopram 10mg mane

Giles Cullity was reviewed on the 11 December 2013 at Eastern Community Mental Health Service.

At Present
He now believes he may have Kennedy Disease – Spinal and bulbar muscular atrophy (SBMA) related to a testosterone receptor problem. He has no fasciculations. Her is also looking at Mild Androgen Insensitivity Syndrome (MAIS). He is worried about a perceived lack of body hair and some other feminine features.

Giles is still under home detention. He has a directions hearing in late January. He is still spending all his time working on his computer project but he is going slowly. He does not have any visitors. He has limited interactions with his family.

At present he said his mood is normal. He is not depressed. His concentration is good. He is able to maintain interest and gain enjoyment. His energy levels are fair. His motivation is reasonable. He does not feel hopeless, worthless or guilty. He denies suicidal ideation or intent. He denies any intent to harm others.

He denied any other psychotic symptoms. He denies hallucinations. He denied thought insertion, thought withdrawal, thought broadcast or thought echo. He denied delusions of reference from TV or radio. He denied any persecutory beliefs or religious beliefs.

He continues to have about 9 hours sleep. His appetite was normal. His weight is stable at 79kg. Height 182cm. BMI is 24. His bowels are regular. His libido is normal.

Assessment
Giles seems relatively stable at present with his mood and thought processes. He continues to be preoccupied with health concerns and perceived muscle problems.

Management Plan
1. He will continue his current treatment
2. He may see his GP re health concerns
3. He will be reviewed in 8 weeks

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Philip Kneebone
Consultant Psychiatrist

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Adelaide Metro Mental Health Directorate

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10 February 2014

Dr Wong
Tusmore Surgery
103 Tusmore Avenue
Tusmore SA 5065

Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Paliperidone 3mg nocte
Citalopram 10mg mane

Giles Cullity was reviewed on the 10 February 2014 at Eastern Community Mental Health Service with his father Garrett and Inge. Giles is still under home detention.

At Present
There have been numerous issues with the courts and several appearances before Judge Kelly. Judge Kelly has requested his parents not supervise his medication!!!! His parents have been happy to supervise his medication although acknowledge he missed a few doses. The judge has said unless sit can be assured that he has medication he will end up in custody later this week. He does not desire to take medication voluntarily although would accept it as part of bail conditions.

Giles is a bit vague about the charge but this does appear to have changed several times. He understands the concept of jail, instructing his lawyers and does not want to use a mental impairment defence. In regard to his crime he reports he did not want to back down as he was worried about his own safety.

He still has concerns about his health – Kennedy's Disease. He will see GP about this. He continues to be fixated on his computer programme. He is still spending all his time working on his computer project but he it is going more slowly than he would like. He has limited interactions with his family.

His behaviour has been largely controlled. He was angry at one of the court appearances.

At present he said his mood is normal. He is not depressed. His concentration is good. He is able to maintain interest and gain enjoyment. His energy levels are fair. His motivation is reasonable. He does not feel hopeless, worthless or guilty. He denies suicidal ideation or intent. He denies any intent to harm others.

He denies hallucinations. He denied thought insertion, thought withdrawal, thought broadcast or thought echo. He denied delusions of reference from TV or radio. He denied any persecutory beliefs or religious beliefs.

He continues to have about 9 hours sleep in short bursts to get away from his work. His appetite was normal. His weight is stable at 79kg. Height 182cm. BMI is 24. His bowels are regular. His libido is normal.

He does not take any illicit substances.

Mental State Examination
He was a tall man with short hair dressed casual clothes. He has slowed movements but there was no evidence of agitation or tremor. His conversation was normal in rate. His affect was

restricted but reactive and appropriate at times. He denied any perceptual abnormalities. His cognition was grossly intact. His insight was limited and his judgement was impaired. Rapport was superficial.

Assessment
Giles seems relatively stable at present despite the Judge's concerns. He continues to be preoccupied with health concerns and perceived muscle problems which appear delusional. Given the issues and his lack of insight he has been placed on a Level 1 CTO.

Management Plan
1. He will be changed to paliperidone oral and if tolerant then to depot
2. Review in 2 days
3. CTO Level 1 applied

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely



Dr Philip Kneebone
Consultant Psychiatrist

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12 February 2014

12 FEB 2014

Dr Wong
Tusmore Surgery
103 Tusmore Avenue
Tusmore SA 5065

Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Paliperidone 3mg nocte for another week
Paliperidone depot 150mg stat and then 75mg 4 weekly from next week
Citalopram 10mg mane

Giles Cullity was reviewed on the 12 February 2014 at Eastern Community Mental Health Service with his father Garrett. Giles is still under home detention and now a Level 1 CTO.

At Present
He has been taking paliperidone 3mg daily for 2 days without side effects or deterioration in mood.

Assessment
Giles seems relatively stable at present. He continues to be preoccupied with health concerns and perceived muscle problems which appear delusional. He has had no problems with oral paliperidone.

Management Plan
1. He will have paliperidone oral 3mg until next week
2. He will have paliperidone 150mg today and then 75 mg every 4 weeks from next Wednesday
3. Side effects and long term risks discussed
4. Review in 2 weeks
5. CTO Level 2 paperwork completed

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Philip Kneebone
Consultant Psychiatrist

Government of South Australia
SA Health, Operations
Adelaide Metro Mental Health Directorate

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26 February 2014

Dr Wong
Tusmore Surgery
103 Tusmore Avenue
Tusmore SA 5065

Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Paliperidone depot 75mg 4 weekly
Citalopram 10mg mane

Giles Cullity was reviewed on the 26 February 2014 at Eastern Community Mental Health Service with his father [redacted] Giles is still under home detention and now a Level 1 CTO.

At Present
He was seen with his father Garrett. Giles is still under home detention and now a Level 1 CTO. His next court appearance is in May 2014.

He has had two injections. He has no major side effects. He report being a little sluggish.

He is reasonably content but is feeling a bit less productive. His concentration is variable. He denies any major worries.

His sleep is a bit harder. His appetite has been good. His weight is 78kg. Ht 182cm. . BMI 24.

He continues to be focussed on his plans to the exclusion of interest in his family or the community.

Mental State Examination
He was a tall man with short hair dressed casual clothes. He has slowed movements but there was no evidence of agitation or tremor. His conversation was normal in rate. His affect was restricted but reactive and appropriate at times. He denied any perceptual abnormalities. His cognition was grossly intact. His insight was limited and his judgement was impaired. Rapport was superficial.

Assessment
Giles seems relatively stable at present.

Management Plan
1. He will have paliperidone 75 mg every 4 weeks
2. CTO hearing next week
3. Review in 3 weeks

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Philip Kneebone
Consultant Psychiatrist

Government of South Australia
SA Health, Operations
Adelaide Metro Mental Health Directorate

19 March 2014

Eastern Community Mental Health

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Dr Wong
Tusmore Surgery
103 Tusmore Avenue
Tusmore SA 5065

Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Paliperidone depot 75mg 4 weekly
Citalopram 10mg mane

Giles Cullity was reviewed on the 19 March 2014 at Eastern Community Mental Health Service with his father Garrett. Giles is still under home detention and now a Level 2 CTO.

At Present
His next court appearance is in May 2014. There is still debate as whether he is fit to give instruction. He has not completed the Centrelink issues.

He has attended some meals with the family and cooked a few meals. He has his laptop back from the police. He thinks his computer project should take another few months to a year. He expects to get a sizeable amount of money.

He is reasonably content or comfortable. His concentration is good. He denies any major worries.

His sleep is a bit more regular despite his complaints. His appetite has been good. His weight has increased to 79kg. Ht 182cm. BMI 24.

He has had some blood tests for some illnesses which have not confirmed his hypotheses.

Mental State Examination
He was a tall man with short hair dressed casual clothes. There was no evidence of agitation or tremor. His conversation was normal in rate. His affect was restricted but reactive and appropriate at times. He denied any perceptual abnormalities. His cognition was grossly intact. His insight was limited and his judgement was impaired. Rapport was slightly improved.

Assessment
Giles seems to have improved recently on depot medication.

Management Plan
1. He will continue paliperidone 75 mg every 4 weeks
2. Review in 6 weeks

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Philip Kneebone
Consultant Psychiatrist

30 APR 2014

Government of South Australia
SA Health, Operations
Adelaide Metro Mental Health Directorate

Eastern Community Mental Health

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30 April 2014

Dr Wong
Tusmore Surgery
103 Tusmore Avenue
Tusmore SA 5065

Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Paliperidone depot 75mg 4 weekly
Citalopram 10mg mane

Giles Cullity was reviewed on the 30 April 2014 at Eastern Community Mental Health Service with his father Garrett. Giles is still under home detention and a Level 2 CTO.

At Present
He was seen with his father [redacted] His father says he has been stable. He has shown some empathy toward his mother.

Giles has a directions hearing in 2 days.

He has a health care card. He has had no major side effects from the injection. He is tolerating it well.

He has cooked a few meals for the family. He continues to work on his computer project. He is not so positive about whether he will complete the project. He still has overvalued beliefs about his capacity.

His mood is reasonably content or comfortable. His concentration is good. He denies any major worries. He has been exercising more regularly.

His sleep is a bit more regular but not back to normal, 0200-0800 1000-1200. His appetite has been good. His weight has increased to 80kg. Ht 182cm. BMI 24.

Assessment
Giles seems to have maintained his improvement on depot medication. He continues to have an uncertain trajectory in his legal issues. He seems to have understanding of the legal process he faces.

Management Plan
1. He will continue paliperidone 75 mg every 4 weeks
2. Review in 6-8 weeks

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Philip Kneebone
Consultant Psychiatrist

Government of South Australia
SA Health, Operations
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16 June 2014

Dr Wong
Tusmore Surgery
103 Tusmore Avenue
Tusmore SA 5065

Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Paliperidone depot 75mg 4 weekly
Citalopram 10mg mane

Giles Cullity was reviewed on the 16 June 2014 at Eastern Community Mental Health Service with Inge. Giles is still under home detention and a Level 2 CTO.

At Present
He is still being reviewed for fitness to instruct lawyers for court. He is unsure when his next court case is due.

He has had no major side effects from the injection.

He has very few social contacts. He continues to cook a few meals for the family. He continues to work on his computer project. He is not sure whether he will complete the project. He still has overvalued beliefs. He has an idea for making money via a drum innovation. He has not been playing his drums. His mood is reasonably content. His concentration is good. He denies any major worries. He has been exercising more regularly.

His sleep is a bit more regular but not back to normal. Midnight to 1000. His appetite has been good. His weight has increased to 84kg. Ht 182cm. BMI 25.

Mental State Examination
He was a tall man with short hair dressed casual clothes. There was no evidence of agitation or tremor. His conversation was normal in rate. His affect was restricted but reactive and appropriate at times. He denied any perceptual abnormalities. His cognition was grossly intact. His insight was limited and his judgement was mildly impaired. Rapport was slightly improved.

Assessment
Giles seems to have maintained his improvement on depot medication.

Management Plan
1. He will continue paliperidone 75 mg every 4 weeks
2. Inge will try to organise some activities for him
3. Review in 6-8 weeks

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Philip Kneebone
Consultant Psychiatrist

Government of South Australia
SA Health, Operations
Adelaide Metro Mental Health Directorate

Eastern Community Mental Health

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172 Glynburn Road
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29 August 2014

Dr Wong
Tusmore Surgery
103 Tusmore Avenue
Tusmore SA 5065

Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Paliperidone depot 75mg 4 weekly
Citalopram 20mg evening

Giles Cullity was reviewed on the 29 August 2014 at Eastern Community Mental Health Service with [redacted] Giles is still under home detention and a Level 2 CTO.

At Present
Seen with [redacted]

He has pleaded guilty this morning to "assault causing serious harm with intent to cause serious harm". There are mixed messages re outcome of this re charges - ? suspended sentence or gaol term. There is a submissions hearing on 4 November. There is a sentencing hearing before the end of the year.

He is upset that his desire to work on his computer plan has lessened. He says his thinking has not been clear enough to work on his project.

He has very few social contacts. He continues to cook a few meals for the family. He continues to work on his computer project. He is not sure whether he will complete the project. He still has overvalued beliefs. He has an idea for making money via a drum innovation. He has not been playing his drums.

His mood has been a bit low. His concentration is poor. His energy levels are low. His motivation is low. He is finding it hard to sustain interest or gain enjoyment. He is worried about the legal issues and being on medication. He has some remorse and guilt re the results of the assault. He feels a little hopeless. He does not feel worthless. He denies feeling suicidal. He however says he would feel this way if he had to take medication the rest of his life.

His sleep is regular and excessive. He lies in bed for extended periods of time. His appetite has been good. His weight is stable at 84kg. Ht 182cm. BMI 25.

Mental State Examination
He was a tall man dressed in casual clothes with his hoody pulled over his head. He was downcast. There was no evidence of agitation or tremor. His conversation was normal in rate. His affect was flat with minimal reactivity. He denied any perceptual abnormalities. His cognition was grossly intact. His insight was limited and his judgement was mildly impaired. Rapport was fair.

Assessment
Giles has become depressed as a result of his depot medication and increased preoccupation with his legal predicament. He has pleaded guilty. He denies suicidal intent.

Management Plan

1. Increase citalopram to 20mg evening
2. Review 3/9/2104
3. Depot dose to be reviewed 3/9/2014

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely



Dr Philip Kneebone
Consultant Psychiatrist

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3 September 2014

Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Paliperidone depot reduce to 50mg 4 weekly
Citalopram 20mg evening

Giles Cullity was reviewed on the 3 September 2014 at Eastern Community Mental Health Service with [redacted] Giles is still under home detention and a Level 2 CTO.

At Present
He does not believe there has been any change from the increased citalopram. He has not noticed any side effects.

He is still upset as he has been unable to work on his project. He can still think about things but struggles to think of new ideas. He says he has the old information is there but he is not making any progress. He does not have a psychotic interpretation of this and attributes it to the medication.

He has very few social contacts. He occasionally has meals from the family.

He has pleaded guilty – he is hoping for a suspended sentence. He thinks he might be at some risk if he went to gaol. He still thinks he may have androgen insensitivity.

His mood has been a bit low. 3/10. His concentration is poor. His energy levels are low. His motivation is low. He is finding it hard to sustain interest or gain enjoyment. He is worried about the legal issues and being on medication. He has some remorse and guilt re the results of the assault. He feels a little hopeless. He does not feel worthless. He having suicidal ideation or intent but admitted that if he felt this way if he had to take medication the rest of his life. He denies intent to harm others.

His sleep is regular and excessive. He lies in bed for extended periods of time. His appetite has been good. His weight is stable at 84kg. Ht 182cm. BMI 25.

Assessment
Giles remains depressed and has limited reactivity. He attributes this to medication. He denies suicidal intent.

Management Plan
1. He will reduce his depot paliperidone to 50mg im every 4 weeks
2. Review 12/9/2014

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Philip Kneebone
Consultant Psychiatrist

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Adelaide Metro Mental Health Directorate

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12 September 2014

Dear Dr Wong

Re: **Giles CULLITY** **DOB: 22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Paliperidone depot 50mg 4 weekly
Citalopram 20mg evening

Giles Cullity was reviewed on the 12 September 2014 at Eastern Community Mental Health Service with [redacted] Giles is still under home detention and a Level 2 CTO.

At Present
Seen with [redacted] for part of interview.

He has not noticed any effects from the increased citalopram. He has been concerned about sexual side effects on medication.

He is still upset as he has been unable to work on his project.

He has very few social contacts. He occasionally has meals from the family.

He was not cooperative today as he is angry about the whole court/medication/ treatment issue. He wants to convince everyone he does not have schizophrenia.

He was unwilling to discuss symptoms in any detail due to his feeling frustrated. He has aims for the future including finishing his computer project.

He denied suicidal ideation or intent. He denied homicidal ideation or intent.

Mental State Examination
He was a tall man dressed in casual clothes with his hoody pulled over his head. He was downcast. There was no evidence of agitation or tremor. His conversation was normal in rate. His affect was flat with minimal reactivity. He denied any perceptual abnormalities. His cognition was grossly intact. His insight was limited and his judgement was mildly impaired. Rapport was fair.

Assessment
Giles remains depressed and has limited reactivity. He attributes this to medication. He denies suicidal intent or homicidal intent. He agreed to make contact if this changes.

Management Plan
1. Continue present treatment
2. Review 3 weeks with depot injection

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Philip Kneebone
Consultant Psychiatrist

07 OCT 2014

Government of South Australia
SA Health, Operations
Adelaide Metro Mental Health Directorate

3 October 2014

Eastern Community Mental Health

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Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Paliperidone depot 50mg 4 weekly
Citalopram 20mg evening

Giles Cullity was reviewed on the 3 October 2014 at Eastern Community Mental Health Service with [redacted] Giles is still under home detention and a Level 2 CTO.

At Present
His father has noticed some improvement. He had a hearing at the magistrates court 24 September 2014 re assault police charge.

He has been unable to work on his project. He has realised his project may not be patentable. He realises he may have to have another source of income. He has thought of doing something with drums.

He has very few social contacts. He occasionally has meals from the family.

He was cooperative today but he is frustrated about the whole court/medication/ treatment issue.

His mood has been a bit despondent but he has not been irritable. He denied suicidal ideation or intent. He denied homicidal ideation or intent.

He is sleeping 0200 to 1300. His appetite is normal. His weight is 82kg. Ht 182cm. BMI 25.

Mental State Examination
He was a tall man dressed in casual clothes. There was no evidence of agitation or tremor. His conversation was normal in rate. His affect was reactive. He denied any perceptual abnormalities. His cognition was grossly intact. His insight was fair and his judgement was mildly impaired. Rapport was fair.

Assessment
Giles has improved and is more reactive. He denies suicidal intent or homicidal intent.

Management Plan
1. Continue present treatment
2. Review 6 weeks

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Philip Kneebone
Consultant Psychiatrist

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14 November 2014

Dear Dr Wong

Re: **Giles CULLITY** **DOB: 22/07/1993** **UR: 204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Paliperidone depot 50mg 4 weekly
Citalopram increase to 30mg and then 40mg if required

Giles Cullity was reviewed on the 14 November 2014 at Eastern Community Mental Health Service with [redacted] Giles is still under home detention and a Level 2 CTO.

At Present
Unfortunately Giles has burnt himself on a number of occasions to eradicate some moles. He still has some wounds that are healing. He has promised not to do this again. He has other appearance concerns but would not discuss these and has also agreed to discuss this prior to action.

He had a hearing at the Magistrates Court which was adjourned until 19 December. A Supreme Court Hearing was also adjourned after the lawyers sent a request for a report to the wrong address. Dr Raeside will be writing more reports. The victim impact statement had a negative impact on him. Next Supreme Court hearing later in November.

He has not done any work on his project. He has very few social contacts. He occasionally has meals from the family.

He was cooperative today but he is frustrated about the whole court/medication/ treatment issue.

His mood has been a bit despondent but it is variable. When he is alone he is more depressed. He has not been irritable. He has been exercising. He denied suicidal ideation or intent. He denied homicidal ideation or intent.

He is sleeping 0200 to 1300. His appetite is normal. His weight is 83kg. Ht 182cm. BMI 25.

Assessment
Giles has remained relatively stable but has burnt himself recently to get rid of moles. He denies suicidal intent or homicidal intent. He states he will not do this again.

Management Plan
1. Increase citalopram dose to 30mg and 40mg if tolerated
2. **2 x ECG's ordered – GP to review QTc please while I am on leave**
3. To see GP re skin wounds
4. Review 4 weeks after my leave

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Philip Kneebone
Consultant Psychiatrist

Government of South Australia
SA Health, Operations
Adelaide Metro Mental Health Directorate

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17 December 2014

Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Paliperidone depot 50mg 4 weekly
Citalopram increase to 40mg daily

Giles Cullity was reviewed on the 17 December 2014 at Eastern Community Mental Health Service.

At Present
He has received a 3 year suspended sentence on 4 December 2014 with a one year non parole as he pleaded guilty. He provided a sentencing report. He was not found not guilty by reason of mental impairment. He is also ordered to reside with his parents. If he has any issues he could end up in jail. His family are happy. He sees corrections every week. He is relieved the court process is over although he still has one outstanding charge of assault police. He recognises he would have to take medication as conditions of his sentence or risk gaol. His family are keen to go on a holiday.

He occasionally goes for a walk. He has very few social contacts. He has meals with his family as a sign of gratitude.

He does not believe the medication increase has been of any benefit. He has had ECG's which do not show major QTc prolongation.

He has not done any work on his project. He only gets enjoyment form achievement of goals. He has not removed any more skin lesions and they are slowly healing.

His mood has been a bit despondent. He has not been irritable. He has been exercising. He denied suicidal ideation or intent. He denied homicidal ideation or intent.

He is sleeping 0200 to 1300. His appetite is normal. His weight has increased to 84kg. Ht 182cm. BMI 25.

He denies any psychotic symptoms or abnormal beliefs.

Assessment
Giles remains relatively stable. He denies suicidal intent or homicidal intent.

Management Plan
1. Continue present treatment
2. Review in 6 weeks

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Philip Kneebone
Consultant Psychiatrist

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30 January 2015

Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Paliperidone depot reduce to 25mg every 3 weeks
Citalopram increase to 40mg daily

Giles Cullity was reviewed on the 30 January 2015 at Eastern Community Mental Health Service with his father Garrett for part of interview.

At Present
He remains under Corrections with weekly appointments. He sees DI Miller. He has a 3 year suspended sentence. He went to Perth for a week with his family. He spends time lying in bed or walking around the house. He has not caught up with any friends. He has very few social contacts. He still has meals with his family. He has not done any work on his project. He says he is about 50% He has not removed any more skin lesions.

His CTO lapses soon but he says he will take treatment as directed by his suspended sentence. His father he wants to be safe at home. he is required to live at home as part of his suspended sentence.

He has ongoing concerns about his body and concerns about his androgen receptors. He focuses on his shoulder width, hip width, facial structures and bone structure. He thinks he should be on testosterone. He says this has always been a concern. He identifies as a male and heterosexual.

His mood has been a bit depressed. He tried to bring up his mood a bit in Perth. He has not been irritable. He has been exercising. He denied suicidal ideation or intent. He denied homicidal ideation or intent. He is sleeping 0200 to 1300. His appetite is normal. His weight is stable at 84kg. Ht 182cm. BMI 25.

He denies any psychotic symptoms or abnormal beliefs. He feels paralysed in his thinking and attributes this to the paliperidone.

Assessment
Giles remains relatively stable. He denies suicidal intent or homicidal intent.

Management Plan
1. Reduce paliperidone to 25mg every 3 weeks
2. CTO to lapse and treatment under legal orders
3. Recommend see an endocrinologist
4. Continue present treatment
5. Review in 6 weeks

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Philip Kneebone
Consultant Psychiatrist

06 MAR 2015

Government of South Australia
SA Health, Operations
Adelaide Metro Mental Health Directorate

6 March 2015

Eastern Community Mental Health

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Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Pallperidone depot 25mg every 3 weeks
Citalopram increase to 40mg daily

Giles Cullity was reviewed on the 6 March 2015 at ECMHS with his father [redacted]

At Present
Seen with [redacted]

He remains under Corrections with fortnightly appointments. He sees DI Miller. He has a 3 year suspended sentence. His CTO has lapsed. He has been going for a few walks. He has not caught up with any friends. He has very few social contacts. He still has meals with his family. He is cooking 3 times per week. He has not done any work on his project. He has some aims for the next year.

He has ongoing concerns about his body and androgen receptors. He has not seen a doctor.

His mood has been a marginally improved 6/10. He has not been irritable. His concentration remains impaired. He has been exercising. He denied suicidal ideation or intent. He denied homicidal ideation or intent.

He is sleeping less - 0200 to 1200. His appetite is normal. His weight has increased to 86kg. Ht 182cm. BMI 26. His bowels are normal.

He denies any psychotic symptoms or abnormal beliefs.

Mental State Examination
He was a tall man dressed in casual clothes. There was no evidence of agitation or tremor. His conversation was normal in rate. His affect was more reactive. He denied any perceptual abnormalities. His cognition was grossly intact. His insight was fair and his judgement was mildly impaired. Rapport was fair.

Assessment
Giles remains relatively stable. He denies suicidal intent or homicidal intent.

Management Plan
1. Continue pallperidone 25mg every 3 weeks
2. He has been encouraged to seek referral to an endocrinologist
3. Recommend psychologist
4. Review in 6-8 weeks

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Philip Kneebone
Consultant Psychiatrist

Government of South Australia
SA Health, Operations
Adelaide Metro Mental Health Directorate

14 April 2015

Eastern Community Mental Health

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Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Paliperidone depot 25mg every 3 weeks
Citalopram 40mg daily

Giles Cullity was reviewed on the 14 April 2015 at ECMHS with his father [redacted]

At Present
He remains under Corrections with fortnightly appointments. He sees DI Miller. He has a 3 year suspended sentence. He is not on a CTO.

He has been a bit more motivated and active. He says he is less depressed.

He has seen an endocrinologist Anthony Roberts. He has accepted his androgen receptors may be within the normal range. He is still awaiting further advice re a test and its costs.

He has been going for infrequent walks. He has not caught up with any friends. He has very few social contacts.

He still has meals with his family. He is cooking 2 times per week. He has not done any work on his project.

His mood is a bit less depressed and a bit more motivated. 6/10. He has not been irritable. His concentration remains impaired. He has been exercising occasionally. He denied suicidal ideation or intent. He denied homicidal ideation or intent.

He is sleeping less - 0200 to 1300. His appetite is normal. His weight has increased to 88kg. Ht 182cm. BMI 27. His bowels are normal.

He denies any psychotic symptoms or abnormal beliefs.

Assessment
Giles remains relatively stable. His mood and activity levels have improved marginally. He denies suicidal intent or homicidal intent.

Management Plan
1. Continue paliperidone 25mg every 3 weeks
2. Recommend psychologist
3. Review in 6-8 weeks
4. Care coordinator to encourage increased activity

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Philip Kneebone
Consultant Psychiatrist

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SA Health, Operations
Adelaide Metro Mental Health Directorate

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2 June 2015

Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Paliperidone depot 25mg every 4 weeks
Citalopram 40mg daily

Giles Cullity was reviewed on the 2 June 2015 at ECMHS with his mother.

At Present
Seen with mother [redacted]. She says he has been helpful and not disruptive.

He remains under Corrections with fortnightly appointments. He sees Di Miller. He has a 3 year suspended sentence. He is not on a CTO.

He has not been as active. He says he is less depressed.

He has seen an endocrinologist Anthony Roberts. He has accepted his androgen receptors may be within the normal range. He is still awaiting further advice re a test and its costs.

His mood is only fair. He has not been irritable. His concentration is variable. He has been exercising occasionally. He denied suicidal ideation or intent. He denied homicidal ideation or intent.

His sleeping has been 0100 to 1200. His appetite is normal. His weight has increased to 91kg. Ht 182cm. BMI 28. His bowels are normal.

He denies any psychotic symptoms or abnormal beliefs.

Assessment
Giles remains relatively stable. His mood and activity levels have improved marginally. He denies suicidal intent or homicidal intent. His behaviour has been the casue of his problems. he

Management Plan
1. Reduce paliperidone to 25mg every 4 weeks
2. Review in 6-8 weeks
3. Care coordinator to encourage increased activity

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Philip Kneebone
Consultant Psychiatrist

Government of South Australia
SA Health, Operations
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15 July 2015

Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Paliperidone depot 25mg every 4 weeks
Citalopram reduce to 30mg daily

Giles Cullity was reviewed on the 15 July 2015 at ECMHS.

At Present
He remains under Corrections with fortnightly appointments. He sees Di Miller. He has a 3 year suspended sentence. He is not on a CTO.

He has not done anything more about the androgen receptors. He has been looking for typing work on line to assist payment for a fine.

He stays at home most of the time. He has been going for infrequent walks. He has not caught up with any friends – he is a bit of a loner. He still has meals with his family. He is cooking 2 times per week. He has not done any work on his project.

He says he is a little depressed. His mood is only fair. He has not been irritable. His concentration is variable. He has been exercising occasionally. He denied suicidal ideation or intent. He denied homicidal ideation or intent.

His sleeping has been 0100 to 1200. His appetite is normal. His weight has increased to 94kg. Ht 182cm. BMI 28. His bowels are normal.

He denies any psychotic symptoms or abnormal beliefs.

Mental State Examination
He was a tall man dressed in casual clothes. There was no evidence of agitation or tremor. His conversation was normal in rate. His affect was more reactive. He denied any perceptual abnormalities. His cognition was grossly intact. His insight was fair and his judgement was mildly impaired. Rapport was fair.

Assessment
Giles remains relatively stable. His mood and activity levels have improved marginally. He denies suicidal intent or homicidal intent. he is keen to reduce the citalopram.

Management Plan
1. Reduce citalopram to 30mg daily
2. Review in 6-8 weeks
3. Care coordinator to encourage increased activity

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Philip Kneebone
Consultant Psychiatrist

27 AUG 2015

Government of South Australia
SA Health, Operations
Adelaide Metro Mental Health Directorate

26 August 2015

Eastern Community Mental Health

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Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Paliperidone depot 25mg every 4 weeks
Citalopram reduce to 20mg daily

Giles Cullity was reviewed on the 26 August 2015 at ECMHS with his father [redacted]

At Present
He has not found any work but may deliver the Courier. He has been out occasionally.

He stays at home most of the time. He has been walking infrequently. He has not caught up with any friends – he is a bit of a loner. He still has meals with his family. He is cooking 2 times per week. He has not done any work on his project.

He says he is a little depressed. His mood is only fair. He has not been irritable. His concentration is variable. He has been exercising occasionally. He denied suicidal ideation or intent. He denied homicidal ideation or intent.

His sleeping has been 0100 to 1200. His appetite is normal. His weight is stable at 94kg. Ht 182cm. BMI 28. His bowels are normal.

He denies any psychotic symptoms or abnormal beliefs.

Mental State Examination
He was a tall man dressed in casual clothes. There was no evidence of agitation or tremor. His conversation was normal in rate. His affect was more reactive. He denied any perceptual abnormalities. His cognition was grossly intact. His insight was fair and his judgement was mildly impaired. Rapport was fair.

Assessment
Giles remains relatively stable. His mood and activity levels have improved marginally. He denies suicidal intent or homicidal intent. He is keen to reduce the citalopram further.

Management Plan
1. Reduce citalopram to 20mg daily
2. Sick certificate
3. Review in 8 weeks
4. Care coordinator to encourage increased activity

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Philip Kneebone
Consultant Psychiatrist

20 OCT 2015

Government of South Australia
SA Health, Operations
Adelaide Metro Mental Health Directorate

Eastern Community Mental Health

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20 October 2015

Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Paliperidone depot 25mg every 4 weeks
Citalopram 20mg daily

Giles Cullity was reviewed on the 20 October 2015 at ECMHS with his father [redacted]

At Present
He reports that Giles has been a bit flat especially immediately after reducing his citalopram dose. He has not been motivated.

He remains under Corrections with fortnightly appointments. He sees Di Miller. He has a 3 year suspended sentence. He has not been placed on Centrelink payments.

He has not had much contact with Inge.

He has not heard back from The Eastern Messenger. He has been out occasionally.

He stays at home most of the time. He has been walking infrequently. He has not caught up with friends. He still has meals with his family. He is cooking 2 times per week. He has not done any work on his project as he does not feel creative. He has done some reading on line.

He says he is a little depressed. His mood is only fair. He has not been irritable. His concentration is variable. He has been exercising occasionally. He denied suicidal ideation or intent. He denied homicidal ideation or intent.

His sleeping has been 0100 to 1200. His appetite is normal. His weight has reduced to 89kg. Ht 182cm. BMI 27. His bowels are normal.

He denies any psychotic symptoms or abnormal beliefs. He still has some concerns that if he exercises excessively he will become weaker.

Assessment
Giles remains relatively stable. His mood and activity levels have deteriorated marginally. He denies suicidal intent or homicidal intent.

Management Plan
1. Review in 3 weeks
2. Care coordinator to encourage increased activity

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Philip Kneebone
Consultant Psychiatrist

DATE: / 11 NOV 2015 /
DR ON LEAVE: WW
DR COVERING: JEC

Government of South Australia
SA Health, Operations
Adelaide Metro Mental Health Directorate

Eastern Community Mental Health

Office Address
Eastern Community Mental Health Centre
172 Glynburn Road
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11 November 2015

Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Paliperidone depot 25mg every 4 weeks
Citalopram 20mg daily

Giles Cullity was reviewed on the 11 November 2015 at ECMHS.

At Present
[redacted] reported by email that Giles has been a bit more motivated and interacting more with the family. He remains under Corrections with fortnightly appointments. He sees DI Miller. He has a 3 year suspended sentence. He is not on Centrelink payments.

He has not had much contact with his care coordinator. He will be going to Perth for Christmas and has some acknowledgement that this will be good.

He has been going out every day for a walk. He stays at home most of the time. He has been walking infrequently. He has not caught up with friends. He has meals with his family. He is cooking 2 times per week.

He has done some work on matters related to his project. He has done some reading on line and gains some pleasure. He has started online learning on logic and has been researching art and history. He denies any psychotic symptoms or abnormal beliefs.

He says his mood is a little better. He has not been irritable or distressed butr a little frustrated. His concentration is variable. He denied suicidal ideation or intent. He denied homicidal ideation or intent.

His sleeping has been 0100 to 1200. His appetite is normal. His weight is stable at 89kg. Ht 182cm. BMI 27. His bowels are normal.

Assessment
Giles remains relatively stable. His mood and activity levels have again improved marginally. He denies suicidal intent or homicidal intent.

Management Plan
1. Review in 6 weeks after my leave
2. He will consider going on oral antipsychotic if possible under corrections conditions
3. Care coordinator to encourage increased activity

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Phillip Kneebone
Consultant Psychiatrist

Government of South Australia
SA Health, Operations
Adelaide Metro Mental Health Directorate

16 December 2015

Eastern Community Mental Health

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Tel: (08) 7425 5555
Fax: (08)7425 5556

Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Paliperidone depot 25mg every 4 weeks
Citalopram 20mg daily

Giles Cullity was reviewed on the 16 December 2015 at ECMHS with [redacted]

At Present
He is about to go to Perth for a holiday with family. He remains under Corrections with fortnightly appointments and is about to get a new worker. He has a 3 year suspended sentence. He is not on Centrelink payments. He has not applied for Centrelink DSP.

He has been going out every day for a walk. He has not caught up with friends. He is cooking 2 times per week. He remains preoccupied with his appearance and his perceived endocrine abnormality and this restricts his socialisation.

He has done limited work on matters related to his project. He has done some reading on line and gains some pleasure. He has started online learning on logic and has been researching art and history. He denies any psychotic symptoms but still admits some abnormal beliefs.

He says his mood is a bit depressed. He has not been irritable or distressed but a little frustrated. His concentration is variable. He denied suicidal ideation or intent. He denied homicidal ideation or intent.

His sleeping has been 0100 to 1200. His appetite is normal. His weight is stable at 86kg. Ht 182cm. BMI 26. His bowels are normal.

Mental State Examination
He was a tall man dressed in casual clothes. There was no evidence of agitation or tremor. His conversation was normal in rate. His affect was more reactive. He denied any perceptual abnormalities. His cognition was grossly intact. His insight was fair and his judgement was mildly impaired. Rapport was fair.

Assessment
Giles remains relatively stable. He continues to express concerns re his appearance. He denies suicidal intent or homicidal intent.

Management Plan
1. Review in 4 weeks
2. He will consider going on oral antipsychotic if possible under corrections conditions
3. Recommend apply for DSP

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Philip Kneebone
Consultant Psychiatrist

13 JAN 2016

Government of South Australia
SA Health, Operations
Adelaide Metro Mental Health Directorate

Eastern Community Mental Health

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13 January 2016

Dear Dr Wong

Re: **Giles CULLITY** DOB: **22/07/1993** UR: **204742**

Diagnosis Psychosis ? Schizoaffective Disorder

Medication Paliperidone depot ceased
Risperidone 2mg nocte
Citalopram 20mg daily

Giles Cullity was reviewed on the 13 January 2016 at ECMHS with [redacted]

At Present
He went to Perth for 11 days with his family. He feels a bit more energetic after this. He remains under Corrections with fortnightly appointments. He has served one year of a 3 year suspended sentence (see conditions). He is not on Centrelink payments. He has not applied for Centrelink DSP.

He has been going out most days for a walk. He has not caught up with friends. He is cooking 2 times per week. He is preoccupied with his appearance and his perceived endocrine abnormality but has not sought further investigation or clarification.

He has done little work on his project. He has done some reading on line and gains some pleasure. He is still doing some online learning. He has still been researching art and history. He denies any psychotic symptoms but has abnormal beliefs.

He says his mood is a bit depressed. He has not been irritable or distressed. His concentration is variable. He denied suicidal ideation or intent. He denied homicidal ideation or intent.

His sleeping has been 0200 to 1400. His appetite is normal. His weight has increased to 88kg. Ht 182cm. BMI 27. His bowels are normal.

Assessment
Giles remains relatively stable. He continues to express concerns re his appearance. He denies suicidal intent or homicidal intent. he is aware of consequences if he is non compliant with oral medication.

Management Plan
1. ~~He is keen to go on oral medication~~ – cease depot
2. ~~Change to 2mg risperidone nocte.~~
3. ~~Recommend apply for DSP~~ – paperwork
4. Review 6 weeks

If you wish to discuss any of the above or other issues please contact me on 0401997230.

Yours sincerely

Dr Philip Kneebone
Consultant Psychiatrist